

Updated S2 K Guidelines for the Management of Bullous Pemphigoid Initiated by the European Academy of Dermatology and Venereology (EADV)

Abstract

Background

Bullous pemphigoid (BP) is the most common autoimmune subepidermal blistering disease affecting the skin and mucous membranes. It primarily occurs in the elderly, presenting with pruritus and localized or generalized bullous lesions. A subset of patients may present with non-bullous manifestations such as excoriations, prurigo-like lesions, eczematous changes, or urticarial erythema. BP is significantly associated with neurological disorders, carries high morbidity, and markedly impairs quality of life.

Objectives and Methodology

The Autoimmune Blistering Diseases Task Force of the European Academy of Dermatology and Venereology updated the management guidelines for BP based on emerging clinical evidence, diagnostic tools, and therapeutic interventions. Recommendations are either evidence-based or derived from expert consensus, with documentation of agreement among task force members.

Results

Treatment selection depends on disease severity and patient comorbidities.

- **High-potency topical corticosteroids** are recommended as first-line

therapy whenever feasible.

- **Oral prednisone at 0.5 mg/kg/day** is an alternative, particularly for more extensive disease.
- In cases of corticosteroid contraindications or treatment resistance, **immunosuppressive agents** such as methotrexate, azathioprine, mycophenolate mofetil, or mycophenolic acid may be used.
- **Doxycycline and dapsone** have uncertain efficacy; however, they may be considered, especially in patients who cannot tolerate oral corticosteroids.
- For refractory disease, **B-cell-depleting therapy (e.g., rituximab)** and **intravenous immunoglobulins (IVIG)** are potential options.
- Emerging biologics such as **omalizumab** and **dupilumab** have shown promising results in early studies.

The final guideline version was endorsed by several patient organizations, emphasizing patient-centered care.

Conclusions

These updated guidelines provide evidence- and expert opinion-based recommendations for the management of bullous pemphigoid, designed to support clinical decision-making.

Introduction

Bullous pemphigoid (BP) is the most prevalent autoimmune subepidermal blistering dermatosis. It is characterized by tissue-bound and circulating autoantibodies targeting two components of hemidesmosomes: **BP180 (BPAG2 or type XVII collagen)** and **BP230 (BPAG1e, epithelial isoform)**. These proteins are critical for dermal-epidermal adhesion, and their autoimmune disruption leads to subepidermal blister formation.

BP predominantly affects individuals over 70 years of age, with a mean diagnosis age of approximately 80 years in Europe. The disease is marked by severe pruritus and cutaneous blistering, significantly impairing quality of life. It is associated with considerable morbidity and a two- to threefold increased mortality compared to age- and sex-matched controls.

The annual incidence ranges from 6 to 43 per million, with rising trends observed over the past two decades. This increase may be attributed to greater recognition of non-bullous presentations, a growing prevalence of neurological conditions (such as dementia and Parkinson's disease) linked to BP, and increased use of medications implicated in BP induction—particularly **gliptins** and **immune checkpoint inhibitors**. These drug associations underscore the need for systematic evaluation of medication history and consideration of drug discontinuation in affected patients.

Updates to the management consensus were prompted by new randomized and open-label clinical trials evaluating immunomodulatory agents and biologics, advances in diagnostic methodologies, and a heightened focus on patient-reported outcomes such as quality of life. Shared decision-making between clinicians and patients is now recognized as a key component of treatment planning and should be integrated into routine practice.